

Diabetes & Perimenopause

What actually changes in managing type 1 diabetes after 40, what helps, and how to be taken seriously by your care team.

Lena Filatova

Founder of the School of Diabetes and of AI-powered health & wellness solutions. Living with type 1 diabetes for 23 years

Inside this workbook

- Why your management got harder, and why it isn't your fault
- 30+ signs of perimenopause, including ones you'd never suspect
- A two-week log you can take to your appointment
- What to say to your doctor, and six questions worth asking
- How to get proper NHS care when you aren't being heard
- Four tools that genuinely changed things for me
- UK organisations for further information
- Sources, rules and law you can cite at an NHS appointment to get help

How to use this workbook

This workbook exists to help you recognise the signs of perimenopause, gather information worth having, and get proper support from your doctor. I'd suggest printing some of the pages and filling them in by hand — it's the filled-in pages that make the conversation with a clinician productive. Here are the steps I'd recommend.

1 Tick your signs in the first chapter

What you tick becomes your symptom list at the appointment.

2 Fill in and print the logs on pages 15 and 16

Ideally across two different phases of your cycle, rather than two quiet weeks in a row.

3 Complete the appointment preparation page

Write down the three things that get in the way of your life most, and one specific number for each.

4 Take the printouts with you

Paper on the desk changes the conversation more than any description can. You can show page 25 with the sources alongside it.

PLEASE NOTE. This workbook is educational and does not replace a consultation with a doctor. Everything in it is designed to get you to your appointment prepared, not to treat yourself. Always speak to a clinician before making decisions about your health.

Notes

You're not imagining it

I've lived with type 1 diabetes since 2003, and I always coped reasonably well with what it threw at me: complicated home cooking, exercise, eating while travelling, stressful situations. But as I approached 40, methods I had spent decades refining stopped working the way they always had.

At 38 I started having panic attacks, and a very strange allergic reaction: hives across my whole body, appearing daily, like clockwork, in the second half of the evening. I knew something systemic was happening in my body, but neither I nor my doctors could work out what. Every test was normal. I was eating the same way and exercising just as regularly.

Between 39 and 41 I worked hard on my insulin sensitivity: adding and changing exercise, working on my mental health. I collected data consistently, analysed it, adjusted my doses, and kept asking doctors what was going on and what to do about it. They had no answer beyond «you're doing wonderfully».

At 42 more symptoms arrived that, alongside normal test results, were impossible to address. Every doctor reassured me that I was fine, and that it was too early to be thinking about menopause because my cycle was regular.

But I had gathered evidence of my body behaving so unreasonably that it was becoming hard to ignore. For example: in the morning I make one coffee with soya milk, about 4g of carbohydrate. My blood sugar climbs to 18 mmol/L.

At the gym, the same story: glucose rising steeply during the session and after it, and a real fight to bring it back into range. Resistance training does raise glucose, so part of this was expected and I had prepared for it. What I hadn't expected was how long it now took to come down.

This isn't a failure of your management, and you aren't imagining it. It's biology that the medical world is only beginning to take seriously.

When good numbers cost too much

I asked my diabetes team what was happening. They looked at my readings and said: all good, you're being too hard on yourself. But I knew exactly what those good numbers had cost me – far more effort and far more hunger than the year before. On work trips I was skipping meals rather than risk a spike in the middle of a conference or an event. Any strong emotion, good or bad, took me to 17 or 18 mmol/L.

I started cancelling plans with friends constantly, because the anticipation alone pushed my blood sugar up and I'd have to stay home to bring it back down.

Eating became so difficult that I developed a fear of meals: every time, I was anxious that I wouldn't be able to manage a perfectly healthy, precisely calculated plate of food. I was hungry all the time.

When I raised it with my endocrinologist again, he told me they had no training in women's health for this stage of life. Because my cycles were still regular, he said I wasn't in perimenopause and there was nothing to treat. I should simply keep counting and wait longer before eating.

So I decided to take the problem apart piece by piece. I rang my GP and asked whether anything could be done about the perimenopause symptoms I had. I was lucky: the first doctor I reached happened to specialise in hormone replacement therapy (HRT), and she confirmed that all of my symptoms were indications for HRT at my age. For the first time I felt understood, and not out of my mind. I felt normal.

The mechanism nobody explained to me

When oestrogen and progesterone fluctuate in perimenopause, insulin sensitivity moves with them.² The same meals that used to keep you steady now push you much higher, and glucose rises more easily across the board. Month to month the hormones are produced unpredictably, so one cycle everything works and the next it doesn't.³ Swings like that are hard to catch on a blood test.

Adjusting my doses took a while: I had discussions with other local doctors at the practice, because some of them had no background in diabetes and no sense of how insulin sensitivity connects to female hormones at this stage.

It's widely assumed that if a woman has no hot flushes and a regular cycle, she isn't in perimenopause yet. That isn't so: the international criteria for staging reproductive ageing (Stages of Reproductive Aging Workshop, STRAW+10) are built on changes to the cycle, and hot flushes aren't part of the diagnostic criteria at all — they usually appear later.¹ There are many other little-known symptoms, changes in insulin sensitivity among them.

In the end it all worked, and I got my replacement therapy.

On the first day I used an oestrogen and progesterone patch, my insulin sensitivity came back within a few hours. That night I went to bed at eight and slept straight through without waking, and realised afterwards that I hadn't slept that well in years.

So this workbook is for every woman with diabetes, and for the family and friends who want to support her through this. It's what I would have wanted to be given, and to know, at 38.

The longer version of this story, with links to the research: [Perimenopause and diabetes: what's really happening to your blood sugar](#)

Signs that it's your hormones, not your effort

None of these is a diagnosis on its own. Together, and especially if they appeared in the last year or two as you approached 40 or after, they are worth taking to an appointment. Tick what you recognise: you'll need it on page 18.

BLOOD SUGAR AND DIABETES MANAGEMENT

☐ **Doses that worked for years are now not enough, or not always enough**

With no other explanation: no bigger portions, less movement, stress or illness.

☐ **The same food and the same activity behave differently month to month**

One cycle your usual breakfast is manageable, the next it isn't.

☐ **Emotions move your glucose more than they used to**

Good news or bad, you're 3–4 mmol/L higher and slow to come back down.

☐ **You are working much harder for the same result**

The numbers still look good, but what they cost you has gone up.

☐ **Glucose takes longer to come back into range after exercise**

The rise during resistance training is expected. How long it now lasts is not.

☐ **Overnight and morning numbers are creeping up**

The morning fight starts earlier and needs bigger corrections.

CYCLE AND REPRODUCTIVE SYSTEM

☐ **Shorter cycles**

Periods arriving every 21–24 days instead of 28.

☐ **Longer cycles**

Cycles stretching past 35 days, or a month missed altogether.

☐ **Heavier bleeding (menorrhagia)**

Flooding, large clots, or bleeding through products far quicker than usual.

☐ **Scant bleeding**

Very light spotting that barely needs a pad.

☐ **Erratic timing**

Unpredictable start dates with no pattern you can plan around.

☐ **Prolonged bleeding**

Periods lasting longer than seven consecutive days.

☐ **Spotting between periods**

Bleeding or brown discharge mid-cycle.

☐ **Severe cramping (dysmenorrhoea)**

New or markedly worse pelvic pain.

☐ **Worsening premenstrual syndrome (PMS)**

Breast tenderness, bloating, fatigue, irritability and aches noticeably stronger than before.

CHAPTER ONE

CYCLE AND REPRODUCTIVE SYSTEM (CONTINUED)

- ☐ **Phantom periods**
Cramping, bloating and every PMS symptom, with no bleeding.
- ☐ **Vaginal dryness**
Tissue thinning and drying as oestrogen falls.
- ☐ **Vaginal atrophy**
Loss of elasticity, a sense that the vagina is narrower or shorter.
- ☐ **Pain during sex (dyspareunia)**
Burning, soreness or tearing sensations. This is treatable, often with local oestrogen.
- ☐ **Vulval itching and burning**
Persistent irritation with no infection behind it.

HOT FLUSHES AND TEMPERATURE REGULATION

- ☐ **Classic hot flashes**
A sudden wave of heat spreading across the face and chest.
- ☐ **Night sweats**
Severe enough to change your clothes or the bedding.
- ☐ **Cold flashes straight after a flush**
Shivering, teeth-chattering chills.
- ☐ **Daytime chills for no reason**
Freezing cold in a warm room.
- ☐ **Sudden sweating without heat or exertion**
Perspiration with no trigger at all.
- ☐ **Facial flushing**
Visible redness across the neck, cheeks and chest.
- ☐ **Heat intolerance**
Warm weather or a hot bath has become unbearable.
- ☐ **Clamminess**
A persistent sticky, cold feeling on the skin.

NEUROLOGICAL AND COGNITIVE

- ☐ **[Brain fog](#)**
A general sense of mental cloudiness and slowed thinking.
- ☐ **Short-term memory lapses**
Forgetting why you walked into a room, misplacing keys.
- ☐ **Word-retrieval problems**
Common words won't come and you stall mid-sentence.
- ☐ **Poorer concentration**
Hard to focus on reading, work or conversation.

CHAPTER ONE

NEUROLOGICAL AND COGNITIVE (CONTINUED)

- ☐ **Spatial disorientation**
Clipping door frames, dropping things, mild clumsiness.
- ☐ **Dizziness**
Lightheaded, unsteady, or brief episodes of vertigo.
- ☐ **Tinnitus**
Ringing, buzzing or hissing in one or both ears.
- ☐ **Hormonal migraines**
Severe throbbing headaches tied to cyclical drops in oestrogen.
- ☐ **Tension headaches**
Persistent pressure around the forehead and temples.
- ☐ **Sensory overload**
Loud noise, bright light or crowds drain you quickly.
- ☐ **Phantom smells**
Usually smoke, or something indefinably unpleasant that isn't there.
- ☐ **Formication**
The sensation of insects crawling on or under the skin.
- ☐ **Electric shock sensations**
A sharp zap under the skin, often a second before a flush.
- ☐ **Tingling in the extremities (paraesthesia)**
Pins and needles in fingers, toes, hands, feet.
- ☐ **Numbness**
Temporary loss of sensation in small patches of skin.

MOOD AND MENTAL HEALTH

- ☐ **Sudden irritability**
Snapping at small things, patience gone in an instant.
- ☐ **Unprovoked anger**
Rage or frustration that is hard to control.
- ☐ **Generalised anxiety**
A low-level dread running under ordinary tasks.
- ☐ **Panic attacks**
Sudden overwhelming episodes of terror with physical symptoms.
- ☐ **Health anxiety**
New or obsessive worry that something is seriously wrong.
- ☐ **Low mood**
Persistent sadness, flattened emotions, a sense of emptiness.
- ☐ **Depression**
An existing depression worsening, or a first episode.

CHAPTER ONE

MOOD AND MENTAL HEALTH (CONTINUED)

- ☐ **Crying spells**
Bursting into tears with no obvious trigger.
- ☐ **Loss of confidence**
Suddenly doubting your professional skills, appearance, decisions.
- ☐ **Social withdrawal**
Wanting to shut out friends, family and any gathering.
- ☐ **Mood volatility**
Cycling between happiness, anger and sadness within hours.
- ☐ **A sense of impending dread**
The feeling that something terrible is about to happen.
- ☐ **Loss of joy (anhedonia)**
The things you used to love give you nothing.
- ☐ **Depersonalisation**
Feeling detached from your own body, as if watching yourself.

SLEEP

- ☐ **Trouble falling asleep**
Lying awake for hours.
- ☐ **Waking at 2 or 3am and not getting back off**
The most typical hormonal sleep pattern.
- ☐ **Early waking**
Awake hours before the alarm, wide awake and exhausted at once.
- ☐ **Unrefreshing sleep**
Waking as tired as when you went to bed.
- ☐ **Unbearable night-time hunger**
Hard to fall asleep until you've eaten.
- ☐ **Unusually vivid dreams**
Intense, strange or emotionally draining.
- ☐ **Frequent nightmares**
Distressing enough to break your sleep.
- ☐ **New snoring or pauses in breathing (sleep apnoea)**
New in onset or clearly worse.
- ☐ **Restless legs syndrome (RLS)**
An irresistible urge to move your legs the moment you lie down. The link with menopause is described but mixed: most women with RLS report it worsening afterwards, though hormone therapy does not reliably help.

MUSCLES, JOINTS AND BONE

- ☐ **Morning joint stiffness**
Knees, hips or fingers rigid and creaky on waking.

CHAPTER ONE

MUSCLES, JOINTS AND BONE (CONTINUED)

- ☐ **Deep joint pain (arthralgia)**
Aching and throbbing without arthritis.
- ☐ **Muscle aches**
Generalised soreness like flu without the flu.
- ☐ **Muscle weakness**
Harder to carry shopping or open a jar.
- ☐ **Frozen shoulder**
Severe stiffness and pain in the shoulder joint. One of the most common and least known signs.
- ☐ **Plantar fasciitis**
Sharp stabbing pain in the heel, worst first thing.
- ☐ **Tendonitis**
Inflamed, painful tendons, especially wrists and Achilles.
- ☐ **Lower back pain**
New or noticeably worse across the lumbar region.
- ☐ **Loss of bone density (osteopenia)**
Bone thins faster as oestrogen falls. You won't feel it; a DEXA scan shows it.

HEART AND CIRCULATION

- ☐ **Heart palpitations**
A sudden racing, pounding or fluttering heartbeat.
- ☐ **Skipped beats (ectopic beats)**
The sensation of the heart pausing for a moment.
- ☐ **Blood pressure spikes**
Sudden swings, or a general rise in resting pressure.
- ☐ **Dizziness on standing up quickly**
Postural hypotension, unrelated to a low blood sugar.
- ☐ **Cold hands and feet**
Poorer peripheral circulation.
- ☐ **Bruising easily**
Fragile capillaries and marks you can't account for.
- ☐ **Swollen ankles**
Fluid retention and poorer drainage.

SKIN, HAIR AND NAILS

- ☐ **Extreme skin dryness**
Rough and flaky, needing heavy moisturiser.
- ☐ **Loss of skin elasticity**
Skin thinning, sagging and wrinkling faster than you expected.

CHAPTER ONE

SKIN, HAIR AND NAILS (CONTINUED)

- ☐ **Hormonal acne**
Deep, painful cystic breakouts along the jawline and chin.
- ☐ **Itchy skin with no rash**
Itching all over, including inside the ears and across the shoulder blades.
- ☐ **Scalp hair thinning**
Diffuse loss, or a parting that keeps widening.
- ☐ **Brittle nails**
Splitting, peeling and cracking at nothing.
- ☐ **Ridged nails**
Deep vertical lines that weren't there before.
- ☐ **Facial hair (hirsutism)**
Coarse dark hairs on the chin, upper lip and jaw.
- ☐ **Body hair loss**
Thinning of pubic, underarm or leg hair.
- ☐ **Changed body odour**
The same deodorant has stopped working the way it did.
- ☐ **Sudden skin sensitivity and new allergies**
Hives or reactions to fabrics, soaps or sunlight you were fine with before.

DIGESTION AND METABOLISM

- ☐ **Fat redistributing to the abdomen**
The same number on the scales, a different shape, different clothes.
- ☐ **Weight creeping up with no change to how you eat**
What always held your weight steady has stopped working.
- ☐ **Abdominal bloating**
Clothes tight by the evening.
- ☐ **New food intolerances**
Dairy, gluten or spice suddenly causing upset.
- ☐ **Histamine intolerance**
Wine, aged cheese or yesterday's leftovers bringing on a rash, congestion or headache.
- ☐ **Nausea**
Mild but persistent waves, like morning sickness.
- ☐ **Constipation**
A markedly more sluggish gut.
- ☐ **Diarrhoea or irritable bowel syndrome (IBS)**
Sudden urgency, loose stools, cramping after eating.
- ☐ **Heartburn and acid reflux**
Acid coming up more often than it used to.

CHAPTER ONE

DIGESTION AND METABOLISM (CONTINUED)

☐ **Excessive gas**
More of it, and more painful, than was ever normal for you.

☐ **Changed sense of taste**
Food tasting metallic, bitter, or simply flat.

BLADDER

☐ **Urinary frequency**
Needing to go far more often through the day.

☐ **Waking at night to urinate (nocturia)**
Several times a night, specifically for this.

☐ **Stress incontinence**
Leaking when you cough, sneeze, laugh or train.

☐ **Urge incontinence**
A sudden uncontrollable need that you can't hold.

☐ **Recurrent urinary tract infections**
Frequent painful UTIs as the lining thins.

☐ **Bladder pain (interstitial cystitis)**
Burning on passing urine with no infection on testing.

☐ **Weakened pelvic floor**
A heavy, dragging sensation low down.

MOUTH, EYES, BREASTS AND GENERAL

☐ **Burning mouth syndrome**
A burning pain on the tongue, palate or lips.

☐ **Bleeding gums**
Inflammation and bleeding when you brush. Your dentist may notice before you connect it.

☐ **Dry mouth**
Not enough saliva to swallow or speak comfortably.

☐ **Dry eyes**
Gritty, burning, light-sensitive, tired.

☐ **Vision changes**
Blurring, or an unexpected change in your prescription.

☐ **Breast tenderness**
Deep throbbing or shooting pain in one or both breasts.

☐ **Change in breast shape and size**
Loss of glandular tissue altering the contour.

☐ **Altered libido**
A sharp drop or, sometimes, a sudden temporary spike.

Total physical exhaustion that a good night's sleep doesn't touch.

Write the number here and carry it across to your appointment preparation page.

Two things you'll be told often that simply aren't true: that regular periods rule out perimenopause, and that without hot flashes it hasn't started. Neither is correct.¹ My cycles were regular throughout and I never had a single flush.

A word about mood. If low mood, anxiety or a sense of detachment from yourself persists for weeks, or gets in the way of living, don't wait for the next routine appointment. That deserves to be raised with a doctor separately and sooner, whether or not hormones turn out to be behind it.

Anything else you've noticed that isn't on the list

Your two-week log

Two weeks of structured data will change an appointment more than any description can. Print this page and fill it in by hand: the aim isn't perfect record-keeping, it's a pattern a clinician can see in ten seconds.

Day	Cycle day	Fasting glucose	Total daily insulin	Time in range %	Sleep (hrs / quality)	Most noticeable symptom
1						
2						
3						
4						
5						
6						
7						
8						
9						
10						
11						
12						
13						
14						

Make the contrast visible. If you can, log across two different phases of your cycle rather than two quiet weeks in a row. The difference between the weeks is the evidence – two identical weeks tell your team far less.

Symptom diary and two-week summary

The numbers show the pattern; the symptoms show the cost. Write the symptoms that affect you into the left-hand column, mark each day they occur, and at the end of the fortnight count how many days each one took from you.

	1	2	3	4	5	6	7	8	9	10	11	12	13	14

Two-week summary

Highest total daily insulin

Lowest total daily insulin

Difference between them

Average time in range, week 1

Average time in range, week 2

Cycle day when it was hardest

That difference is your strongest argument. If your total daily dose varies between the two weeks by more than 15–20 per cent on the same food and the same activity, that is the number to open the conversation with on the next page.

Notes

What to say at your appointment

I was sent home enough times with «you're doing wonderfully» and «you look well» — in other words, with the position that I was imagining it. So it isn't only about collecting clear data; it's about framing your concern, and the effect the symptoms have on your daily life, in a way that lands. Clinicians respond to change over time and to specifics: the same facts, put differently.

INSTEAD OF

«My blood sugars have been odd lately.»

TRY

«Over the last six months my total daily insulin has gone from 32 to 45 units on the same food and the same training. Here's my log and my online data.»

INSTEAD OF

«I think I might be in perimenopause?»

TRY

«I'm recording a cyclical pattern in my insulin resistance, and a list of symptoms I've kept in a diary. I'd like to discuss whether hormonal changes are contributing.»

If you aren't being heard

Don't give up. Keep gathering information and keep communicating your concern. If you need to, ask for a referral to an HRT specialist within your practice.

And use the rules that already exist. The most useful one here: since September 2025, England has Jess's Rule, under which a third presentation with the same symptoms means the GP team should rethink the diagnosis rather than repeat the previous conclusion.⁷ The full list of rules you can cite is on page 20.

WORDING THAT WORKS:

«This is my third visit with the same symptoms. I'd like us to rethink the diagnosis under Jess's Rule, and I'd like it recorded in my notes whether hormonal changes were considered as a factor.»

Notes

Appointment preparation page

Fill this in the night before and take it with you along with your log. Ten minutes here is worth more than an hour of explaining in the room.

What I've come in with today

Very briefly, in one sentence: the symptoms that trouble you most.

How much this affects daily life

For example, whether the symptoms limit your work, or affect your daily routine.

What I've already tried in order to improve things

What I want from this appointment

One sentence: tests, a referral to a specialist, information resources, and so on.

How quickly I need this to change

Whether it needs solving urgently, or the symptoms aren't badly limiting and can wait.

If you're told perimenopause is impossible because your cycles are regular and you have no flushes, politely ask for that reasoning to be entered in your notes, or ask for a second opinion. Regular cycles and no flushes didn't stop me getting HRT in the end – it just took a specialist willing to listen.

What was said at the appointment

Fill this in in the room, or straight afterwards. If a question wasn't answered, put a dash: that's information too, and it's exactly what you'll need at the next visit.

- 1 Does anyone on this team have training in women's health after 40?

- 2 Could falling oestrogen explain the rise in my insulin requirement?

- 3 What exactly would you need to see from me to consider hormonal changes as a factor?

- 4 Is there a reason not to refer me to a perimenopause specialist?

- 5 How do I tell perimenopause symptoms apart from hypoglycaemia symptoms?

- 6 Can we agree a review date rather than leaving this open-ended?

What we agreed

Review date _____

Referral: to whom, and when _____

What I need to do before next time _____

What went in my notes about hormonal changes _____

Notes

How to get proper NHS care when you aren't being heard

Most women don't get help not because it doesn't exist, but because they don't know what it's called. These are the rules you can cite out loud, and every one of them takes a minute to verify online.

Jess's Rule: three presentations and we rethink

If you come back a third time with the same or escalating symptoms, the GP team should rethink the diagnosis rather than repeat the previous conclusion: think again, take a colleague's view and, if needed, refer on. It's an initiative of the Department of Health and Social Care (DHSC) and NHS England, backed by the Royal College of General Practitioners (RCGP), in force since September 2025.⁷ It isn't law and it isn't an automatic right to investigation, but it is an official expectation on practices.

Say this: «This is my third visit with the same symptoms. I'd like the diagnosis rethought under Jess's Rule.»

After 45, diagnosis is made on symptoms, not on blood tests

The National Institute for Health and Care Excellence (NICE) guideline NG23 says it plainly: do not use laboratory tests to diagnose perimenopause or menopause in people aged 45 and over. Between 40 and 45, a follicle-stimulating hormone (FSH) test may be considered where there are symptoms alongside a change in cycle. So «your bloods are normal, therefore it isn't perimenopause» runs against the guideline.⁹

Say this: «As I understand NICE NG23, at my age this is a clinical diagnosis. Can we work from the symptoms?»

The right to choose where you're referred

In England, for a first referral to a consultant-led service you have a legal right to choose any clinically appropriate provider, and even a particular team under a named consultant. This is the mechanism people use to reach a menopause clinic rather than whichever service is nearest.¹⁰

Say this: «I'd like to use my right to choose and be referred to a menopause clinic. This is the one I'd like.»

Where this applies. Jess's Rule and the right to choose apply in England. NICE guidelines apply in England and Wales; Scotland and Northern Ireland have their own equivalents. If you're not in England the principle is the same and the name is different: ask your practice what it's called where you are.

Notes

How to get proper NHS care, continued

Your record belongs to you

Under the Data Protection Act 2018 and UK GDPR you have a legal right to see what has been written about you. The quickest route is the NHS App; if what's there is incomplete, put a written Subject Access Request (SAR) to the practice. It's the best way to check whether your concerns were actually recorded and whether hormonal changes were considered.

Read your notes after every appointment, not once a year.

A second opinion: you can ask, but it isn't guaranteed

There is no legal right to a second opinion, but you can always ask, and practices usually accommodate it. If you're refused, ask for the refusal and the reasoning to go in your notes. That changes the tone of the conversation sharply.

If that doesn't help, contact your local Patient Advice and Liaison Service (PALS), then make a formal complaint, and only after that go to the Parliamentary and Health Service Ombudsman (PHSO).

HRT at a fixed annual cost

The HRT Prescription Prepayment Certificate (HRT PPC) costs £19.80 as a one-off and covers an unlimited quantity of certain HRT medicines for 12 months, against £9.90 per prescription item. So if you collect HRT three or more times a year, the certificate pays for itself.¹¹

Check your particular medicine is on the covered list before you buy.

Your annual diabetes review is a lever too

Diabetes UK sets out 15 checks and services you're entitled to every year. Emotional and psychological support is named among them, as is the opportunity to discuss sexual problems. That's a legitimate basis for putting women's health on the agenda of your annual review rather than waiting for a separate appointment.¹²

Say this: «I'd like this year's annual review to include a conversation about hormonal changes.»

One sentence worth learning by heart: «Please record in my notes that I raised this, and what was decided.» It costs nothing, it sounds courteous, and it works better than any argument.

Notes

Four tools that genuinely worked

This is what worked for me, in order of how much it mattered. Every decision about your management belongs to you and your doctor. My job here is to make sure you know what exists and what to ask about.

1 The HRT conversation

The biggest lever I found. My GP wasn't initially convinced that hormones affect insulin sensitivity – something I considered so fundamental that any clinician would know it, since every hormone influences every other. This isn't guesswork: the effect of hormone therapy on glycaemia specifically in women with diabetes has already been pulled together into a systematic review you can cite.⁴ It took time, discussion and, of course, a specialist who understood the area. But you do need a doctor to prescribe – don't put yourself on hormones. On supplements, my view is this: the things sold over the counter (herbs, tinctures, homeopathy) are about as effective at managing female hormones as they are at managing insulin.

Start here: ask for a referral, not a prescription. Regular cycles are not an obstacle. [What's happening with HRT prescribing](#)

2 Resistance training two or three times a week

The most reliable non-drug tool there is: muscle improves how your body handles glucose, protects bone and steadies energy better than any single supplement.⁵ It's a proven way to hold on to muscle and bone as you enter perimenopause. If you've never done resistance work, start as soon as you can. Live with the paradox: glucose often rises during a session and immediately afterwards, while the effect over a few weeks is firmly in the other direction. Blood sugars become steadier and easier to handle if you train consistently.

Start here: two sessions a week, basic compound movements, and judge the result by the month rather than by the session.

Notes

Four tools, continued

3 More protein and fibre, and no fasting

At this stage, against a background of fluctuating oestrogen, cortisol packs fat onto the abdomen. Fasting, stress and swings in glucose and insulin only help that process along. So drop the harsh diets and the prohibitions, which can worsen perimenopause symptoms and drive episodic binges on sugar and calories. Collagen and muscle start to be lost faster, which is why eating enough protein matters so much now. Do follow your doctor's advice on how much protein is right for you, but look at the newer research and ask whether that advice is worth revisiting.

If I've no appetite or no time to eat, I blend protein powder with vegetables and berries and add ground flaxseed. It tastes good, it fills me up, and it raises blood sugar very slowly. [How much protein you actually need after 40](#)

4 Sleep and stress: the cheapest lever, and the first to disappear

Perimenopause wrecks sleep, poor sleep worsens your management, and high glucose wrecks your rest and raises cortisol. In women, even moderate sleep loss over a few weeks reduces insulin sensitivity independently of any change in weight,⁶ and in postmenopausal women the same effect shows up after just four nights.⁸ It's a closed loop, and it's the one nobody usually asks about, so raise it yourself. With broken sleep, joint pain, apathy and swinging blood sugars, even training gets harder. I noticed that too. But simply deciding to sleep well may not work: treat this symptom seriously and push for a solution.

Start here: log your sleep alongside your glucose on page 15, so the connection becomes visible rather than theoretical.

All of these tools work better together. Give your body time to adjust — somewhere in the region of six to twelve months — keep watching what changes, and stay in touch with your doctor in case something needs adjusting.

Notes

UK organisations for further information

If you're looking for where to start, these organisations hold the most relevant information for women with diabetes going through perimenopause – and here's what to ask the NHS for.

Diabetes UK

Britain's largest diabetes charity: there's a helpline and a well-moderated support forum where this particular combination comes up often. Useful when you need to be sure you're not the only one.

diabetes.org.uk

Women's Health Concern (WHC) and the British Menopause Society (BMS)

WHC is the patient arm of the BMS, the professional society for menopause. Their factsheets are the ones I'd take to an appointment: written by clinicians, referenced, and hard to wave away.

womens-health-concern.org · thebms.org.uk

Jess's Rule (NHS England)

The official page for the «three strikes and we rethink» initiative. If you're going in for the third time with the same symptoms, it's worth having to hand.

[england.nhs.uk · Jess's Rule](https://england.nhs.uk/Jess's+Rule)

NICE NG23 and the NHSBSA

The menopause guideline itself, which you can cite at an appointment, and the page where you buy the HRT PPC for £19.80 a year. Both official, both free to read.

nice.org.uk/guidance/ng23

Read on: the longer version of my story, with links to the research, is on the blog:

lenafilatova.co.uk/blog/perimenopause-diabetes-blood-sugar

Notes

Sources, rules and law you can cite

The numbers match the markers in the text, and the titles are clickable. Don't print all of it: pick the one or two closest to your situation and name them at the appointment alongside your log.

- 1 [Harlow SD et al. Executive summary of the Stages of Reproductive Aging Workshop +10 \(STRAW+10\). Menopause, 2012.](#)
The international criteria for staging perimenopause. Built on changes to the cycle; hot flushes are not among the diagnostic criteria.
- 2 [Menopause management in women with type 1 diabetes mellitus: a narrative review. Endocrine Connections, 2026. doi.org/10.1530/EC-25-0486](#)
A review of what is and isn't known about menopause in type 1 diabetes: falling oestradiol, reduced insulin sensitivity, changes in body composition.
- 3 [Menstrual Cycle, Glucose Control and Insulin Sensitivity in Type 1 Diabetes: A Systematic Review. 2023. PMC9962060](#)
A systematic review of glycaemic variation and insulin requirement across the cycle in type 1 diabetes.
- 4 [Effect of Postmenopausal Hormone Therapy on Glucose Regulation in Women With Type 1 or Type 2 Diabetes: a systematic review and meta-analysis. Diabetes Care, 2023;46\(10\):1866.](#)
What is known about the effect of hormone therapy on glycaemia specifically in women with diabetes.
- 5 [Effect of combined aerobic exercise and resistance training on postmenopausal women with type 2 diabetes: a systematic review and meta-analysis. Gynecological Endocrinology, 2025.](#)
Combined aerobic and resistance work improved the insulin resistance index (HOMA-IR). The study concerns type 2 diabetes; direct evidence in type 1 is still lacking.
- 6 [Zuraikat FM et al. Chronic Insufficient Sleep in Women Impairs Insulin Sensitivity Independent of Adiposity Changes. Diabetes Care, 2024;47\(1\):117.](#)
Cutting sleep by an hour and a half a night for six weeks reduced insulin sensitivity in women, independently of any change in weight.
- 7 [NHS England. Jess's Rule: Three strikes and we rethink. Published 23.09.2025, updated 14.05.2026.](#)
A DHSC and NHS England initiative supported by the RCGP: rethink the diagnosis after a third presentation with the same symptoms.
- 8 [Effect of sleep restriction on insulin sensitivity and energy metabolism in post-menopausal women: a randomised crossover trial. 2023. PMC10191900](#)
Four nights of restricted sleep reduced insulin sensitivity in postmenopausal women.
- 9 [NICE. Menopause: identification and management, NG23. Updated November 2024. nice.org.uk/guidance/ng23](#)
Over 45 the diagnosis is clinical; laboratory tests are not used to identify perimenopause. Between 40 and 45, FSH may be considered where there are symptoms and a change in cycle.
- 10 [NHS England. Patient choice guidance. england.nhs.uk](#)
The legal right to choose your provider and a named consultant's team at a first referral for a consultant-led appointment in England.
- 11 [NHSBSA. NHS Hormone Replacement Therapy Prescription Prepayment Certificate \(HRT PPC\). nhsbsa.nhs.uk](#)
A £19.80 certificate covering certain HRT medicines for 12 months; a single prescription item costs £9.90.
- 12 [Diabetes UK. 15 Healthcare Essentials. diabetes.org.uk](#)
The checks and services a person with diabetes is entitled to each year, emotional support included.

About Lena

I've lived with type 1 diabetes for 23 years and I write about women's health after 40 for people who want evidence rather than reassurance. I have a background in reading research papers and writing health and fitness news, so when I couldn't find answers about diabetes in perimenopause, I started the conversation openly instead of waiting for one.

I'm not a doctor. I'm someone who sat through those appointments, read the research and worked out which questions get answers. What I want is for you to walk into your next appointment better prepared than I was into mine.

Over to you

This workbook arrived in your inbox. Just hit reply and tell me which of the signs you recognised. I read every message, and they decide what I write about next.

lenafilatova.co.uk

This workbook is educational and reflects my personal experience and my reading of the research. It is not medical advice and does not replace a consultation with a qualified clinician. Do not change insulin, medication or hormone therapy on the basis of anything written here: every decision about your management should be made together with your doctor or your diabetes team. © 2026 Lena Filatova · lenafilatova.co.uk